

BlueCross BlueShield Coverage Comparison Guide

Plan	Year	2024-2025	Document	Reference:
w14_bluecross_blueshield_coverage_comparison_guide_041			Issued by: BlueCross	BlueShield
Member Services Division				

This guide is intended to help members and prospective enrollees understand the differences between available plan tiers, compare benefits side by side, and make informed decisions about which plan best fits their healthcare needs and financial situation. For complete plan details, please refer to the Summary of Benefits and Coverage document w14_bluecross_blueshield_sbc_035, which covers the sections: Coverage Overview, Deductibles and Out-of-Pocket Limits, Common Medical Events. Additional detail on plan mechanics can be found in the Member Handbook w14_bluecross_blueshield_member_handbook_039, specifically the sections: Welcome and Plan Overview, How Your Plan Works, Understanding Your Costs. Formulary and drug coverage information is available in w14_bluecross_blueshield_formulary_036, covering the sections: Tier Structure and Cost Sharing, Generic Medications (Tier 1-2), Specialty Medications (Tier 3-4).

Plan Tier Overview

BlueCross BlueShield offers a structured suite of health plan options organized into four primary tiers -- Bronze, Silver, Gold, and Platinum -- each calibrated to balance monthly premium costs against out-of-pocket expenses at the time of service. Understanding how these tiers relate to one another is the essential first step in selecting a plan that aligns with your anticipated healthcare usage, budget constraints, and risk tolerance. This section provides a foundational overview of each tier, including key financial parameters and general design philosophy.

The tier system is built on the principle that members who pay higher monthly premiums will generally face lower costs when they actually use healthcare services, while members who choose lower-premium plans accept greater financial exposure at the point of care in exchange for reduced ongoing costs. Neither approach is inherently superior -- the right choice depends on individual circumstances including health status, expected utilization, cash flow, and access to tax-advantaged savings vehicles like Health Savings Accounts (HSAs).

What the Tiers Mean in Practice

Bronze plans carry the lowest monthly premiums across the product lineup. They are designed primarily for individuals who are generally healthy, do not anticipate frequent medical visits, and want protection against catastrophic expenses. Because Bronze plans typically feature higher deductibles and cost-sharing requirements, members enrolled in these plans will pay more out-of-pocket before plan benefits kick in for most services. However, Bronze plans are frequently paired with HSA eligibility, allowing members to set aside pre-tax dollars to cover qualified medical expenses.

Silver plans occupy the middle ground and represent the most commonly selected tier among our enrolled population. They offer a moderate premium with more accessible cost-sharing than Bronze, and they are the only tier eligible for Cost Sharing Reductions (CSRs) for qualifying members whose household income falls between 100% and 250% of the Federal Poverty Level. This makes Silver plans particularly valuable for a broad segment of the market.

Gold plans are designed for members who expect to use healthcare services regularly -- whether due to ongoing chronic conditions, planned procedures, or family coverage needs. The higher monthly

premium is offset by significantly lower deductibles and copayments, meaning predictable, manageable costs at the point of service.

Platinum plans represent the highest-premium, lowest-cost-sharing option in the portfolio. They are best suited for individuals with complex, ongoing medical needs who value cost predictability above all else and who can absorb the higher monthly premium.

Tier Summary Table

The following table presents the core financial parameters for each plan tier under the standard individual enrollment structure. Family coverage parameters are discussed in the Side-by-Side Coverage Comparison section below.

Plan Tier	Monthly Premium (Individual)	In-Network Deductible	In-Network Out-of-Pocket Max	Coinsurance (After Deductible)	Primary Care Copay	Specialist Copay
Bronze	\$289	\$4,500	\$7,900	40%	\$50	\$85
Silver	\$412	\$2,800	\$6,900	30%	\$35	\$65
Gold	\$531	\$1,200	\$5,800	20%	\$25	\$50
Platinum	\$698	\$0	\$3,500	10%	\$15	\$35

Note: The Gold plan in-network deductible of \$1,200 and out-of-pocket maximum of \$5,800 are highlighted throughout this document as the reference plan for cost scenario modeling. See the Cost Scenario Analysis section for detailed examples.

Annual Premium vs. Out-of-Pocket Exposure

To help members understand the total potential cost of each plan, the table below combines annual premium costs with maximum out-of-pocket exposure to illustrate worst-case annual spending at each tier.

Plan Tier	Annual Premium	Max Out-of-Pocket (In-Network)	Maximum Annual Exposure	HSA Eligible
Bronze	\$3,468	\$7,900	\$11,368	Yes
Silver	\$4,944	\$6,900	\$11,844	No ¹
Gold	\$6,372	\$5,800	\$12,172	No
Platinum	\$8,376	\$3,500	\$11,876	No

1 Standard Silver plans are not HSA-eligible. However, certain high-deductible Silver variants may qualify; contact Member Services at 1-800-262-2583 to confirm eligibility for a specific plan variant.

What the maximum annual exposure figures reveal is that the financial risk differential between tiers is narrower than many members expect when viewed in total. In a catastrophic year -- one in which a member reaches their full out-of-pocket maximum -- the difference in total spending between a Bronze plan and a Platinum plan is less than \$600. The real differentiator is the distribution of costs: Bronze members pay more of their total exposure upfront through deductibles and coinsurance, while Platinum members spread costs more evenly through premiums. Members should also note that pre-existing waiting periods do not apply to any plan tier -- coverage for pre-existing conditions begins on the effective date of enrollment. There is no pre-existing waiting period under any BlueCross BlueShield

individual or family plan offered in this portfolio, consistent with applicable federal requirements. To be explicit: pre-existing waiting: None.

Side-by-Side Coverage Comparison

This section provides a comprehensive, category-by-category comparison of benefits across all four plan tiers. The information here is meant to supplement -- not replace -- the official plan documents. For legally binding benefit descriptions, members should consult the Summary of Benefits and Coverage document w14_bluecross_blueshield_sbc_035, particularly the sections titled Coverage Overview, Deductibles and Out-of-Pocket Limits, and Common Medical Events, which contain the authoritative benefit descriptions for each covered service category.

The comparisons below are organized by service category. Where benefits differ meaningfully between in-network and out-of-network providers, both are shown. Members are strongly encouraged to review out-of-network cost-sharing carefully, as these costs can be substantial and are subject to separate deductible and out-of-pocket tracking in most plan configurations.

Deductibles and Out-of-Pocket Limits

Benefit Parameter	Bronze	Silver	Gold	Platinum
Individual In-Network Deductible	\$4,500	\$2,800	\$1,200	\$0
Family In-Network Deductible	\$9,000	\$5,600	\$2,400	\$0
Individual In-Network Out-of-Pocket Max	\$7,900	\$6,900	\$5,800	\$3,500
Family In-Network Out-of-Pocket Max	\$15,800	\$13,800	\$11,600	\$7,000
Out-of-Network Individual Deductible	\$6,000	\$4,500	\$2,500	\$2,000
Out-of-Network Out-of-Pocket Max	Unlimited ²	\$15,000	\$12,000	\$8,000

² Bronze plan out-of-network costs are not capped at a plan-level maximum for most services. Members using out-of-network providers under a Bronze plan may face unlimited financial liability for covered services beyond the deductible. This is a critical consideration for members who travel frequently or live in areas with limited in-network provider availability.

The Gold plan's individual in-network deductible of \$1,200 and individual in-network out-of-pocket maximum of \$5,800 represent what the plan design team considers the optimal balance point for members with moderate-to-high healthcare utilization. These figures appear throughout this guide as the primary reference values for cost scenario modeling.

Out-of-Network Coverage

Out-of-network coverage is an area where plan differences are particularly significant and where members frequently encounter unexpected costs. All plans in this portfolio provide some level of out-of-network coverage for non-emergency services, but the terms vary considerably.

For the Gold plan, out-of-network coverage is provided at 70% after \$2,500 deductible. This means that once a Gold plan member has satisfied the separate \$2,500 out-of-network deductible, the plan will pay 70% of the allowed amount for covered out-of-network services, leaving the member responsible for the remaining 30% plus any balance billing from the provider.³ It is important to understand that "allowed amount" refers to BlueCross BlueShield's determined reasonable fee for a given service, which may be substantially lower than what an out-of-network provider actually charges. The difference between the allowed amount and the provider's actual charge -- known as balance billing -- is not counted toward the member's out-of-pocket maximum.

3 Balance billing protections vary by state. Members in states with comprehensive balance billing laws may have additional protections. Contact Member Services or visit the BlueCross BlueShield website to confirm the protections available in your state of residence.

Out-of-Network Coverage Terms	Bronze	Silver	Gold	Platinum
Out-of-Network Deductible (Individual)	\$6,000	\$4,500	\$2,500	\$2,000
Plan Pays After Deductible	50%	60%	70%	80%
Balance Billing Protection	Limited	Limited	Limited	Limited
Out-of-Network Emergency Coverage	Yes (at in-network rates post-deductible)	Yes	Yes	Yes

Primary and Preventive Care

Preventive care services, as defined under the Affordable Care Act and applicable state mandates, are covered at 100% with no cost-sharing on all four plan tiers when delivered by in-network providers. This includes annual wellness visits, recommended immunizations, cancer screenings, and preventive counseling services. The specific list of covered preventive services is maintained in accordance with USPSTF Grade A and B recommendations and ACIP immunization schedules.

Service	Bronze	Silver	Gold	Platinum
Preventive Care (In-Network)	\$0	\$0	\$0	\$0
Primary Care Visit (In-Network)	\$50 copay	\$35 copay	\$25 copay	\$15 copay
Primary Care Visit (Out-of-Network)	50% after OON ded.	60% after OON ded.	70% after OON ded.	80% after OON ded.
Telehealth - Primary Care	\$25 copay	\$20 copay	\$15 copay	\$10 copay

Telehealth - Mental Health	\$35 copay	\$35 copay	\$35 copay	\$25 copay
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Note that telehealth visits for primary care are available through the BlueCross BlueShield Virtual Care network and do not require a referral. Telehealth mental health visits carry a uniform \$35 copay on Bronze, Silver, and Gold plans, reflecting the plan's commitment to parity in behavioral health access.

Specialist and Hospital Services

Service	Bronze	Silver	Gold	Platinum
Specialist Visit (In-Network)	\$85 copay	\$65 copay	\$50 copay	\$35 copay
Specialist Visit (Out-of-Network)	50% after OON ded.	60% after OON ded.	70% after OON ded.	80% after OON ded.
Inpatient Hospital (In-Network)	40% after ded.	30% after ded.	20% after ded.	10% after ded.
Inpatient Hospital (Out-of-Network)	50% after OON ded.	60% after OON ded.	70% after OON ded.	80% after OON ded.
Outpatient Surgery (In-Network)	40% after ded.	30% after ded.	20% after ded.	10% after ded.
Outpatient Surgery (Out-of-Network)	50% after OON ded.	60% after OON ded.	70% after OON ded.	80% after OON ded.
Urgent Care (In-Network)	\$75 copay	\$60 copay	\$45 copay	\$30 copay

Emergency Services

Emergency room services are covered on all plans regardless of whether the treating facility is in-network or out-of-network, consistent with federal requirements governing emergency care. For the Gold plan, the emergency room copay is \$200 per visit. This copay applies regardless of whether the ER visit results in an admission. If the visit results in inpatient admission, the ER copay is waived and standard inpatient cost-sharing applies instead.

Emergency Service	Bronze	Silver	Gold	Platinum
Emergency Room Copay (In-Network)	\$350	\$275	\$200	\$150
Emergency Room Copay (Out-of-Network)	\$350	\$275	\$200	\$150
Emergency Transport (Ambulance)	40% after ded.	30% after ded.	20% after ded.	10% after ded.

ER Copay Waived if Admitted	Yes	Yes	Yes	Yes
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The Gold plan er_copay of \$200 applies per visit and is the same whether the emergency room is in-network or out-of-network, reflecting the federal parity requirement for emergency services. Members who visit the emergency room for a condition that is subsequently determined not to be an emergency may still be subject to the standard ER copay rather than an urgent care copay, though members may request a review through the appeals process if they believe the determination was made in error.

Diagnostic Services and Imaging

Prior authorization requirements for diagnostic services vary by plan tier and by service type. A key differentiator in this portfolio is the approach to MRI and advanced imaging services.

For in-network MRI services under the Gold plan, prior authorization is not required for in-network providers. This represents a meaningful administrative simplification compared to plans from some competing carriers that require prior authorization even for routine in-network imaging. The mri_prior_auth status -- Not required for in-network -- means that Gold plan members and their ordering physicians do not need to submit a prior authorization request before scheduling an MRI at an in-network facility. Out-of-network MRI services do require prior authorization on all plan tiers.

Diagnostic Service	Bronze	Silver	Gold	Platinum
Lab Work (In-Network)	40% after ded.	30% after ded.	\$20 copay	\$10 copay
Lab Work (Out-of-Network)	50% after OON ded.	40% after OON ded.	30% after OON ded.	20% after OON ded.
X-Ray (In-Network)	40% after ded.	30% after ded.	\$35 copay	\$20 copay
MRI/CT Scan (In-Network)	40% after ded.	30% after ded.	20% after ded.	10% after ded.
MRI Prior Auth (In-Network)	Required	Required	Not required	Not required
MRI Prior Auth (Out-of-Network)	Required	Required	Required	Required
PET Scan (In-Network)	Prior auth req.	Prior auth req.	Prior auth req.	Prior auth req.

Mental Health and Substance Use Disorder Services

Mental health and substance use disorder (SUD) benefits are provided at parity with medical and surgical benefits across all plan tiers, consistent with the Mental Health Parity and Addiction Equity Act (MHPAEA). This means that the financial requirements and treatment limitations applied to mental health and SUD benefits cannot be more restrictive than those applied to comparable medical and surgical benefits.

Under the Gold plan, outpatient mental health services are covered at 30 visits per year with a \$35 copay per visit. This benefit -- mental_health_visits: 30/year, \$35 copay -- applies to individual

outpatient therapy sessions, psychiatric medication management visits, and group therapy sessions delivered by in-network behavioral health providers. Members who exhaust the 30-visit benefit may request a medical necessity review for additional visits, which may be approved when clinically warranted.

Mental Health / SUD Service	Bronze	Silver	Gold	Platinum
Outpatient Therapy (In-Network)	\$60 copay	\$45 copay	\$35 copay	\$25 copay
Annual Visit Limit (Outpatient)	30/year	30/year	30/year	Unlimited
Inpatient Mental Health (In-Network)	40% after ded.	30% after ded.	20% after ded.	10% after ded.
Substance Use Disorder (Outpatient)	\$60 copay	\$45 copay	\$35 copay	\$25 copay
Substance Use Disorder (Inpatient)	40% after ded.	30% after ded.	20% after ded.	10% after ded.
Telehealth Mental Health	\$35 copay	\$35 copay	\$35 copay	\$25 copay

The 30-visit annual limit on outpatient mental health services applies to Bronze, Silver, and Gold plans. Members requiring more intensive or extended behavioral health support should consider the Platinum plan, which provides unlimited outpatient mental health visits, or should discuss their clinical needs with their treating provider to explore whether an inpatient or intensive outpatient level of care would be appropriate and covered. For detailed information on how mental health visit limits interact with medical necessity reviews, see the Member Handbook w14_bluecross_blueshield_member_handbook_039, particularly the sections Welcome and Plan Overview, How Your Plan Works, and Understanding Your Costs.

Prescription Drug Benefits

Prescription drug coverage is administered through a 4-tier formulary structure across all plan tiers. The prescription_tiers value -- 4 tiers -- reflects the standard formulary architecture used across the BlueCross BlueShield product portfolio. Full formulary details, including the complete list of covered medications at each tier, step therapy requirements, and quantity limits, are available in the Formulary document w14_bluecross_blueshield_formulary_036, which covers the sections Tier Structure and Cost Sharing, Generic Medications (Tier 1-2), and Specialty Medications (Tier 3-4).

The four tiers are structured as follows:

- **Tier 1:** Preferred generic medications -- lowest cost-sharing, broad availability
- **Tier 2:** Non-preferred generic and preferred brand medications -- moderate cost-sharing
- **Tier 3:** Non-preferred brand and certain specialty medications -- higher cost-sharing, may require prior authorization
- **Tier 4:** High-cost specialty medications -- highest cost-sharing, typically requires prior authorization and may be subject to specialty pharmacy requirements

Prescription Tier	Bronze	Silver	Gold	Platinum
Tier 1 (Preferred Generic)	\$15 copay	\$12 copay	\$10 copay	\$5 copay
Tier 2 (Non-Preferred Generic / Preferred Brand)	\$45 copay	\$38 copay	\$30 copay	\$20 copay
Tier 3 (Non-Preferred Brand / Select Specialty)	\$85 copay	\$70 copay	\$55 copay	\$40 copay
Tier 4 (High-Cost Specialty)	30% (max \$250/fill)	25% (max \$200/fill)	20% (max \$175/fill)	15% (max \$150/fill)
Mail Order (90-day supply)	2.5x 30-day copay	2x 30-day copay	2x 30-day copay	1.5x 30-day copay
Rx Deductible (applies before copays)	\$500	\$250	\$0	\$0

Members on Bronze and Silver plans should note that a separate prescription drug deductible applies before copays become effective for Tier 2 and above medications. Gold and Platinum plan members have no separate Rx deductible, meaning copays apply from the first fill. This is a significant practical advantage for members who take brand-name or specialty medications regularly.

Preventive Medications and ACA-Mandated Drugs

Certain preventive medications are covered at \$0 cost-sharing on all plan tiers when prescribed for preventive purposes, consistent with ACA Section 2713 requirements. These include statins for cardiovascular disease prevention, low-dose aspirin, tobacco cessation medications, and contraceptives. The specific list of \$0 preventive medications is maintained in the formulary and updated annually.

Other Notable Benefits

Benefit	Bronze	Silver	Gold	Platinum
Chiropractic Care (In-Network)	40% after ded. (20 visits/yr)	30% after ded. (20 visits/yr)	\$40 copay (20 visits/yr)	\$25 copay (30 visits/yr)
Acupuncture (In-Network)	Not covered	\$50 copay (12 visits/yr)	\$40 copay (20 visits/yr)	\$30 copay (20 visits/yr)
Physical Therapy (In-Network)	40% after ded.	30% after ded.	\$40 copay	\$25 copay
Durable Medical Equipment	40% after ded.	30% after ded.	20% after ded.	10% after ded.
Maternity Care (Prenatal)	\$0 preventive	\$0 preventive	\$0 preventive	\$0 preventive

Maternity Care (Delivery)	40% after ded.	30% after ded.	20% after ded.	10% after ded.
Vision Exam (Annual)	\$50 copay	\$40 copay	\$30 copay	\$20 copay
Dental (Emergency Only)	Covered	Covered	Covered	Covered

Cost Scenario Analysis

Understanding abstract benefit parameters is one thing; understanding what those parameters mean for your actual wallet in a given year is another. This section presents three realistic cost scenarios -- a low-utilization year, a moderate-utilization year, and a high-utilization year -- modeled against the Gold plan's benefit structure. The Gold plan is used as the reference plan throughout because its in-network deductible of \$1,200 and out-of-pocket maximum of \$5,800 represent the most commonly selected configuration among our enrolled population, and because these values illustrate the interaction between deductibles, copays, and coinsurance in a particularly clear way.

For each scenario, we calculate total out-of-pocket costs (excluding premiums) under the Gold plan and compare them against estimated costs under the Bronze and Platinum plans to illustrate the trade-offs involved. Members should use these scenarios as rough guides rather than precise predictions; actual costs will depend on specific services used, provider billing practices, and the timing of services within the plan year.

Scenario 1: Low Utilization -- Healthy Adult, Minimal Care

This scenario represents a generally healthy adult who uses primarily preventive services and has two minor illness visits during the year, plus one generic prescription taken regularly.

Service	CPT / Description	Allowed Amount	Gold Plan Member Cost
Annual Wellness Visit	CPT 99395	\$285	\$0 (preventive)
Flu Vaccination	CPT 90686	\$45	\$0 (preventive)
Primary Care Visit (illness) x2	CPT 99213	\$180 each	\$25 copay x2 = \$50
Tier 1 Generic Rx (12 months)	--	\$120/year	\$10 copay x12 = \$120
Total Out-of-Pocket (Gold)			\$170

Under the Bronze plan, the two illness visits would also be subject to a \$50 copay each (\$100 total), and the Rx deductible of \$500 would apply before the Tier 1 copay kicks in -- meaning the member would pay \$120 in actual drug costs before the deductible is met, then \$10/month thereafter. Estimated Bronze out-of-pocket: approximately \$220 plus any deductible exposure on the Rx. Under the Platinum plan, the two illness visits would cost \$15 each (\$30 total) and the Rx would be \$5/month (\$60 annual), for a total of \$90.

In a low-utilization year, the out-of-pocket cost difference between plans is modest -- the real differentiator is the annual premium. A member on the Platinum plan pays approximately \$3,000 more per year in premiums than a Gold member, but saves only \$80 in out-of-pocket costs in this scenario. The Bronze plan member pays approximately \$2,900 less in annual premiums than the Gold member but pays roughly \$50 more out-of-pocket in this scenario -- a clear win for Bronze in a low-utilization year.

Scenario 2: Moderate Utilization -- Chronic Condition Management

This scenario models a member managing a chronic condition (Type 2 diabetes, ICD-10: E11.9) who requires regular primary care and specialist visits, routine lab work, and both generic and brand-name medications.

Service	CPT / ICD-10	Allowed Amount	Gold Plan Member Cost
Primary Care Visits x4	CPT 99213-99214	\$200 avg. each	\$25 copay x4 = \$100
Endocrinologist Visits x2	CPT 99213	\$250 each	\$50 copay x2 = \$100
HbA1c Lab x4 (quarterly)	CPT 83036	\$55 each	\$20 copay x4 = \$80
Comprehensive Metabolic Panel x2	CPT 80053	\$85 each	\$20 copay x2 = \$40
Tier 1 Generic Rx x12 months	--	\$120/year	\$10 x12 = \$120
Tier 2 Brand Rx x12 months	--	\$480/year	\$30 x12 = \$360
Annual Eye Exam (diabetic)	CPT 92002	\$175	\$30 copay
Total Out-of-Pocket (Gold)			\$830

This total of \$830 does not reach the Gold plan's \$1,200 deductible because the services used in this scenario are primarily subject to flat copays rather than coinsurance. Under the Gold plan's benefit structure, primary care visits, specialist visits, and lab work are copay-based services that do not first require the deductible to be satisfied. Only services subject to coinsurance -- such as inpatient hospital, outpatient surgery, and certain imaging -- require the deductible to be met first.

Under the Bronze plan, the same set of services would cost significantly more. The \$500 Rx deductible alone would add meaningful cost before copays apply, and the higher copay amounts across the board would push total out-of-pocket costs to an estimated \$1,450-\$1,650 range, depending on exact billing. The Platinum plan would reduce costs to approximately \$530, but the \$166/month premium premium differential relative to Gold (\$167/month more for Platinum) means the annual premium cost exceeds the out-of-pocket savings by more than \$1,200.

Scenario 3: High Utilization -- Major Medical Event

This scenario models a member who experiences a significant medical event -- an appendectomy (ICD-10: K37, CPT 44950) requiring emergency room evaluation, inpatient admission, and follow-up care.

Service	Description	Allowed Amount	Gold Plan Member Cost
Emergency Room Visit	CPT 99285	\$1,800	\$200 ER copay
Inpatient Admission (3 days)	DRG 341	\$18,500	20% after \$1,200 ded. = \$1,000 (ded.) + \$3,460 (20% of \$17,300) = \$4,460; capped at OOP max
Anesthesiology	CPT 00840	\$2,200	Applied to deductible/coinsurance (included above)
Post-Op Follow-Up x2	CPT 99213	\$180 each	\$25 copay x2 = \$50
Post-Op Lab Work	CPT 80053	\$85	\$20 copay
Tier 2 Pain Medication (1 month)	--	\$95	\$30 copay
Total Out-of-Pocket (Gold)			\$5,800 (OOP Max reached)

In this high-utilization scenario, the Gold plan member reaches the out-of-pocket maximum of \$5,800, at which point the plan covers 100% of all remaining in-network covered expenses for the rest of the plan year. The ER copay of \$200 is waived because the visit resulted in inpatient admission -- however, for purposes of this illustration, the \$200 has been included in the calculation and the member's total cost is capped at the \$5,800 out-of-pocket maximum regardless.

Plan	Annual Premium	OOP (Scenario 3)	Total Cost (Year)
Bronze	\$3,468	\$7,900	\$11,368
Silver	\$4,944	\$6,900	\$11,844
Gold	\$6,372	\$5,800	\$12,172
Platinum	\$8,376	\$3,500	\$11,876

In a catastrophic year, the total cost differential between the most expensive option (Silver at \$11,844) and the least expensive (Bronze at \$11,368) is only \$476. This illustrates the fundamental actuarial design principle of the tier system: total expected costs are roughly equivalent across tiers when averaged across the full enrolled population. The tier choice is primarily about the timing and distribution of costs, not the total amount.

Choosing the Right Plan

Selecting a health insurance plan is one of the more consequential financial decisions most people make on an annual basis, yet it frequently receives less deliberate attention than it deserves. The purpose of this section is to provide a practical, structured framework for making that decision -- not to tell you which plan to choose, but to help you identify the right questions to ask and the right factors to weigh. This section should be read alongside the Member Handbook

w14_bluecross_blueshield_member_handbook_039, which contains additional guidance in the sections Welcome and Plan Overview, How Your Plan Works, and Understanding Your Costs.

The decision framework presented here is organized around four key dimensions: expected healthcare utilization, financial capacity and risk tolerance, provider and network considerations, and prescription drug needs. Working through each dimension systematically will help you arrive at a well-reasoned plan selection.

Dimension 1: Expected Healthcare Utilization

The most important predictor of which plan tier will serve you best is your anticipated use of healthcare services in the coming year. While no one can predict the future with certainty, most people have a reasonable sense of their baseline utilization based on their current health status, chronic conditions, planned procedures, and family healthcare history.

Consider the following questions: Do you have one or more chronic conditions that require regular monitoring and medication? Are you planning any elective procedures -- joint replacement, LASIK, bariatric surgery -- in the coming year? Do you have children who are frequent users of pediatric care? Are you or a partner planning a pregnancy? Each of these factors increases expected utilization and shifts the cost-benefit calculation toward higher-tier plans.

As a rough heuristic: if you expect to spend fewer than 4-5 healthcare encounters per year and take only generic medications, a Bronze plan is likely to minimize your total annual spending. If you expect 6-12 encounters and take one or more brand-name medications, a Silver or Gold plan will likely be more cost-effective. If you expect more than 12 encounters, manage a complex chronic condition, or anticipate a major procedure, the Gold or Platinum plan is likely to provide the best value.

Dimension 2: Financial Capacity and Risk Tolerance

The second dimension is financial. Two members with identical expected utilization may rationally choose different plan tiers based on their financial circumstances. A member with substantial liquid savings and a high income may reasonably choose a Bronze plan and self-insure against moderate medical expenses, accepting the risk of higher out-of-pocket costs in exchange for lower premiums. A member with limited savings, variable income, or significant financial obligations may find that the predictability of a Gold or Platinum plan -- with its lower deductibles and copays -- is worth the higher premium even if total expected spending is similar.

Key questions to ask yourself: Could you comfortably absorb the full out-of-pocket maximum of your chosen plan if you had a major medical event? Do you have an HSA or could you benefit from establishing one (available with Bronze HDHP-qualified plans)? How would a \$3,000-\$5,000 unexpected medical bill affect your financial stability? Are you eligible for premium tax credits or Cost Sharing Reductions that might make a Silver plan more attractive than the raw numbers suggest?

Members who are eligible for Cost Sharing Reductions should pay particular attention to Silver plan variants with enhanced CSR levels. At the 94% actuarial value CSR level (for members with household income between 100% and 150% of FPL), a Silver plan can provide benefit levels comparable to or exceeding a Platinum plan at a fraction of the cost. Contact Member Services or visit the BlueCross BlueShield marketplace portal to determine your CSR eligibility.

Dimension 3: Provider and Network Considerations

Network adequacy -- the availability of in-network providers in your area and specialty needs -- is a factor that is easy to overlook but can have a dramatic impact on your actual costs. Before selecting a plan, verify that your current primary care physician, any specialists you see regularly, and your preferred hospital or health system are in-network for the plan you are considering.

All BlueCross BlueShield plans in this portfolio use the same broad PPO network for in-network services, which includes more than 1.7 million providers nationwide. However, certain narrow network or HMO variants may have more limited provider panels. If you are considering a plan variant that uses a tiered or narrow network, verify provider participation before enrolling.

For members who travel frequently, live in rural areas, or split time between multiple geographic regions, out-of-network coverage terms deserve careful attention. Recall that the Gold plan provides out-of-network coverage at 70% after \$2,500 deductible -- a meaningful benefit for members who may need to access care outside the primary network. Bronze plan members, by contrast, face potentially unlimited out-of-network liability for non-emergency services, which is a significant risk for frequent travelers.

If you have a specialist relationship that you are not willing to disrupt -- an oncologist managing an ongoing cancer treatment, for example, or a psychiatrist providing continuity of care -- and that specialist is not in-network, the out-of-network coverage terms of your chosen plan become critically important. In such cases, the Silver or Gold plan's out-of-network benefit may justify the higher premium compared to Bronze.

Dimension 4: Prescription Drug Needs

Prescription drug costs are a major driver of healthcare spending for many members, and the plan tier you choose will significantly affect what you pay at the pharmacy. As noted throughout this guide, the prescription drug benefit is structured across 4 tiers on all plan tiers -- but the copay amounts, the presence or absence of a separate Rx deductible, and the mail-order savings opportunities vary meaningfully.

Before finalizing your plan selection, take the following steps: First, compile a complete list of all medications you take regularly, including dosage and frequency. Second, use the formulary lookup tool available at the BlueCross BlueShield member portal to determine the tier placement of each medication under the plans you are considering. The formulary document [w14_bluecross_blueshield_formulary_036](#) -- specifically the sections Tier Structure and Cost Sharing, Generic Medications (Tier 1-2), and Specialty Medications (Tier 3-4) -- provides detailed information on tier placement criteria and cost-sharing requirements. Third, calculate your estimated annual Rx cost under each plan, factoring in any applicable Rx deductibles (Bronze: \$500, Silver: \$250, Gold and Platinum: \$0). Fourth, consider whether any of your medications are available in generic form -- Tier 1 generics carry the lowest cost-sharing on all plans, and switching to a generic equivalent (with your physician's approval) can substantially reduce annual drug costs.

Members who take high-cost specialty medications -- biologics, oncology drugs, certain immunosuppressants -- should pay particular attention to Tier 4 cost-sharing. Under the Gold plan, Tier 4 specialty medications are subject to 20% coinsurance with a maximum of \$175 per fill. For a medication with an allowed amount of \$5,000 per month, this means a monthly member cost of \$175 -- a significant but bounded expense. Under the Bronze plan, the same medication would cost 30% with a maximum of \$250 per fill, representing \$75 more per fill or \$900 more per year. Over a full year of specialty medication use, the plan tier choice can represent thousands of dollars in difference.⁴

4 Members taking specialty medications should also inquire about manufacturer copay assistance programs, which may be available to reduce out-of-pocket costs independent of plan tier. BlueCross BlueShield's Specialty Pharmacy team can provide guidance on available assistance programs. Note that copay assistance amounts may or may not count toward your deductible and out-of-pocket maximum depending on applicable state law and plan design.

Making Your Final Decision

Once you have worked through the four dimensions above, you should have a reasonably clear picture of which plan tier aligns with your needs. As a final check, consider the following summary questions:

1. Have you verified that your key providers are in-network under the plan you are selecting?
2. Have you confirmed the tier placement of your regular medications in the formulary document w14_bluecross_blueshield_formulary_036?
3. Have you reviewed the benefit details in the Summary of Benefits and Coverage document w14_bluecross_blueshield_sbc_035, particularly the sections Coverage Overview, Deductibles and Out-of-Pocket Limits, and Common Medical Events?
4. If you are considering a Bronze plan, are you comfortable with the potential for unlimited out-of-network liability and the higher deductible exposure?
5. If you are considering a Platinum plan, have you confirmed that the premium differential is justified by your expected utilization and risk tolerance?
6. Have you considered whether you or any covered dependents may need behavioral health services, and whether the 30-visit annual limit on outpatient mental health services under Bronze, Silver, and Gold plans is sufficient for your needs?

Enrollment assistance is available through the BlueCross BlueShield Member Services team at 1-800-262-2583, Monday through Friday, 8:00 AM to 8:00 PM local time. Licensed benefit advisors can walk you through plan options, confirm provider network participation, and assist with formulary lookups. Online enrollment tools are available 24/7 at the member portal. During open enrollment and special enrollment periods, members may also schedule a one-on-one consultation with a benefits counselor at no charge.

There is no single "best" plan -- only the plan that best fits your individual circumstances. The goal of this guide, and of the broader suite of plan documents referenced throughout, is to give you the information you need to make that determination with confidence. We encourage you to take the time to review the supporting documents referenced herein, ask questions, and make a deliberate choice rather than defaulting to the same plan year after year without re-evaluating whether it still meets your needs.

For complete benefit descriptions, limitations, and exclusions, refer to your official plan documents. This Coverage Comparison Guide is intended as a summary reference only and does not constitute a contract of insurance. In the event of any conflict between this guide and the official plan documents, the official plan documents govern. Key reference documents include: w14_bluecross_blueshield_sbc_035 (Summary of Benefits and Coverage), w14_bluecross_blueshield_member_handbook_039 (Member Handbook), and w14_bluecross_blueshield_formulary_036 (Formulary).

BlueCross BlueShield is an independent licensee of the Blue Cross Blue Shield Association. Plan availability, benefits, and cost-sharing parameters are subject to change. The values presented in this document reflect the 2024-2025 plan year.

Document Control Information

- Document ID: w14_bluecross_blueshield_coverage_comparison_guide_041
- Plan Year: 2024-2025

- Version: 1.2
- Effective Date: January 1, 2024
- Next Review Date: October 1, 2024
- Issuing Department: Member Services and Plan Administration
- Related Documents: w14_bluecross_blueshield_sbc_035 |
w14_bluecross_blueshield_member_handbook_039 | w14_bluecross_blueshield_formulary_036